

Limited Source Justification (LSJ)
FAR 8.401 & GSAM/R 538.7104-3(b)(1)

1. Acquisition Title: Hospital Quality Reporting (HQR) System Development Services Bridge Contract

Division: HHS/CMS/CCSQ

Contracting Activity: HHS/ASFR/OMAS/SBC-IT1

Author: [REDACTED]

2. Nature and/or Description of the Action being Approved: Centers for Medicare and Medicaid Services (CMS) Center for Clinical Standards & Quality (CCSQ) requests a hybrid Firm Fixed Price/T&M Order against GSA MAS GS-35F-356DA with the contractor listed in the table below. This action is a bridge contract supporting ongoing efforts to support and modernize the Hospital Quality Reporting (HQR) system. This acquisition is conducted under the Federal Supply Schedule (FSS) Program using the ordering procedures established by the General Services Administration found in the [General Services Acquisition Manual/Regulation \(GSAM/R\) at subpart 538.71](#). The Government projects award for May 2026 using fiscal year 2026 Operations and Maintenance funds.

Contractor: Bellese Technologies LLC, 10451 Mill Run Circle, Suite 400, Owings Mills, Maryland 21117-5594 (UEI: HXBBEPUC48F5)				
Schedule Contract Number: GS-35F-356DA				
Performance Period	Dates/Duration	Unit Price/CLIN	Multiplier	Total
Base Period	12 months	[REDACTED]	1	[REDACTED]
Option to Extend Services per FAR Clause 52.217-8	6 months	[REDACTED]	1	[REDACTED]
Estimated Total Dollar Value:				[REDACTED]

3. Description of Supplies and/or Services:

CMS procures agile software development services to maintain and enhance the Hospital Quality Reporting (HQR) system. The HQR system maintainer is responsible for all operations and maintenance (O&M) of the HQR system, ensuring it remains current with evolving user experience requirements, data integration capabilities, and quality reporting program changes. The HQR system underpins CMS's statutory obligation to Section 1886(d)(1)(b) and Section 1862(a)(1)(A) of the Social Security Act that requires CMS to issue accurate and timely payments and penalties to Medicare-participating providers across ten (10) individual hospital quality reporting programs.

Specific HQR system maintainer functions under this contract include:

- Updating the master ICD-10 code list to ensure accurate claims-based measure calculations.
- Opening and closing 71 submission windows across 10 inpatient/outpatient quality reporting program settings to enable Medicare providers to submit measure data for payment-based performance calculations.
- Configuring measure calculation code to reflect changes in measure specifications across multiple reporting, submission, and payment periods.
- Calculating and sharing performance data and Overall Hospital Quality Star Ratings data with Public Reporting teams for quarterly refreshes of provider data on the Medicare.gov Compare site.
- Providing the Veterans Health Administration (VHA) and the Department of War (DoW) with facility-specific performance data on CMS measures, and supporting public reporting of VA and DoW facility data on Medicare.gov.
- Providing facility-specific performance reports to hospitals to support quality improvement activities performed by Quality Improvement Organizations (QIOs).
- Migrating HQR system infrastructure from the CCSQ Cloud to the CMS Hybrid Cloud environment by CY2027.
- Developing a Fast Healthcare Interoperability Resources (FHIR)-based API to enable participating facilities to submit data directly from their Electronic Health Records (EHRs), reducing reliance on manual or paper-based submission and providing real-time feedback on data quality.
- Transitioning inpatient and outpatient program support request processes from paper-based workflows to digital case management workflows integrated with existing HQR system infrastructure.

4. Authority Cited and Rationale: Authority of the FSS Program, Title III of the Federal Property and Administrative Services Act of 1949 (41 U.S.C. 251, *et seq.*); Title 40 U.S.C. 501, Services for Executive Agencies, as implemented at [FAR 8.401](#) and [GSAM/R 538.7104-3\(b\)\(1\)](#).

Applicable Exception: FAR 8.401 and GSAM 538.7104-3(b)(ii) — Only one source is capable of providing the supplies or services required at the level of quality required because the supplies or services are unique or highly specialized.

Pursuant to GSAM 538.7104-3(b)(ii), a limited-sources justification is required when an ordering activity restricts consideration to fewer than the required number of schedule contractors on the basis that only one source is uniquely capable of fulfilling the requirement. This justification satisfies this requirement. The factual basis for the determination that Bellese Technologies LLC is the only source capable of providing HQR system maintainer services at the required level of quality is set forth below.

Rationale For Limited-Source Justification:

A. Only One Source Is Capable of Providing Services at the Required Level of Quality

1. Highly Specialized, System-Specific Technical Knowledge

The Hospital Quality Reporting (HQR) system has architectural implementation, data environment, and operational dependencies that are highly specialized and distinct from any other system.

HQR is a large-scale, custom-developed application that is architecturally and operationally different from other systems maintained under Center for Clinical Standards & Quality (CCSQ) /Information Systems Group (ISG) contracts. Unlike ISG systems built on commercial Platform-as-a-Service (PaaS) technologies—which leverage standardized frameworks and more transferable skill sets—HQR operates on a bespoke infrastructure with system-specific tooling, tightly coupled data pipelines, and complex integrations that have evolved over years of continuous development.

Additionally, the system processes and maintains highly specialized datasets that directly support Medicare payment adjustments, reimbursement determinations, and the public reporting of Hospital Star Ratings on Medicare.gov. These mission-critical outputs require precise, system-specific knowledge of data structures, business rules, and regulatory requirements. As a result, the technical expertise required to operate and maintain HQR is not readily transferable from other ISG systems or contracts.

Accordingly, the system as implemented is unique in its architecture, data, and mission-critical role, necessitating a contractor with deep, incumbent knowledge to ensure continuity of operations and to mitigate unacceptable performance, schedule, and programmatic risk.

Bellese Technologies has served as the HQR system maintainer since 2017, accumulating deep, system-specific expertise that is unique to this program. This expertise includes:

Intimate knowledge of HQR's custom codebase and architecture, including the logic governing measure calculation, data ingestion, and performance rating generation — logic that is not documented in any transferable form at a level of granularity sufficient for a new contractor to assume operations without extended transition support from the incumbent.

Mastery of 71 submission window management across 10 distinct inpatient/outpatient quality reporting program settings, each governed by independent regulatory timelines, measure specifications, and stakeholder requirements. The interdependencies among these submission windows, and the downstream consequences of configuration errors, are understood only through operational experience on the live system.

Expertise in the HQR data ecosystem, including the processing of more than 43 million production files annually received from over 10,000 facilities, and the calculation of Hospital Star Ratings published on Medicare.gov. These calculations involve complex program-specific algorithms and require precise understanding of data provenance, quality tolerances, and validation logic embedded in the HQR system.

End-to-end familiarity with HQR's external data-sharing relationships, including data exchange with the Veterans Health Administration (VHA), the Department of War (DoW), CMS's Public Reporting teams, and QIO contractors — relationships that require not only technical interface knowledge but also an understanding of each partner's data requirements, formats, and timelines.

Institutional knowledge of evolving program requirements, including changes to ICD-10 codes, measure specifications, and quality reporting policies across multiple program cycles. The ability to accurately configure the HQR system in response to these changes depends on deep familiarity with prior configurations and their downstream effects on payment calculations.

No other contractor currently possesses this system-specific knowledge. Any prospective replacement contractor would require an extended transition period to onboard staff, conduct knowledge transfer, gain familiarity with the system architecture and tooling, and understand the program requirements to reach the level of operational proficiency required to independently manage the HQR system without risk of service disruption.

2. Scale and Complexity of Operations Cannot Be Rapidly Replicated

The HQR system is one of the largest quality reporting platforms in the federal government, directly affecting Medicare payment determinations for thousands of hospitals nationwide. The volume, complexity, and regulatory sensitivity of HQR operations distinguish it from all other ISG software development requirements. Specifically:

The system processes data across 10 independent quality reporting programs representing diverse inpatient and outpatient settings, each with separate regulatory requirements, submission calendars, and measure specifications.

Performance ratings generated by the HQR system directly inform Medicare reimbursement decisions, meaning errors or delays carry immediate financial and legal consequences for CMS and participating providers.

The system's Hospital Star Ratings are publicly reported on Medicare.gov on a quarterly basis, creating fixed, legislatively informed publication deadlines that cannot be deferred while a new contractor comes up to speed.

The operational tempo of the HQR system—driven by statutory payment schedules, CMS publication timelines, and provider submission cycles—does not allow for the transition-related learning curve typically associated with onboarding a new contractor. Given the limited period of performance for the bridge contract, a non-incumbent contractor would be unlikely to attain the necessary level of system-specific operational proficiency without introducing substantial performance risk. Such risk includes, but is not limited to, missed regulatory deadlines, inaccuracies in payment determinations, and potential degradation of data integrity, all of which could adversely impact CMS mission outcomes.

3. CMS CIO Directive 25-03 — Personnel Vetting and Logical Access Requirements

Effective July 8, 2025, CMS CIO Directive 25-03 requires that all individuals seeking access to CMS information systems complete enhanced vetting procedures—including background adjudication and fingerprinting—prior to obtaining logical system access. CMS remains fully compliant with this Directive and has established well-defined onboarding and offboarding processes to ensure appropriate access controls are maintained.

However, the time required to complete the mandatory vetting process—currently averaging approximately 1.5 to 2 months—creates a significant onboarding delay before new contractor personnel can obtain system access and begin performing work. Because access to CMS systems and tools is a prerequisite for performing any meaningful HQR system maintenance, development, or operational support, this delay directly impacts the Government's ability to rapidly staff the requirement.

In the context of this bridge contract, these onboarding timelines materially increase the risk associated with transitioning to a new contractor, as a newly awarded vendor would be unable to field a fully operational workforce for an extended period. In contrast, the incumbent contractor maintains a fully vetted workforce of approximately [REDACTED] FTEs with active system access, enabling immediate, uninterrupted performance on day one. This existing access represents a critical capability that supports continuity of operations and mitigates schedule and performance risk during the bridge period.

As the incumbent, Bellese Technologies maintains a fully onboarded, credentialed team of roughly [REDACTED] full-time equivalents (FTEs) with current logical access to the HQR system and all associated tools, in full compliance with CIO Directive 25-03. A new contractor awarded this requirement would need to complete the full CIO Directive 25-03 vetting process for all assigned personnel before any individual could access HQR system environments. Given the volume of personnel required to sustain HQR operations and

the current processing timelines under the enhanced vetting requirements, this onboarding period would create a significant gap in operational coverage that CMS cannot sustain given its statutory payment and reporting obligations.

Bellese ability to staff [REDACTED] with all logical access to systems and tools on day one of the contract period of performance is itself a capability differentiator that no other contractor can replicate within the required timeframe, further supporting the determination that only one source is capable of providing the required services at the required level of quality during the bridge period.

4. Risk of Operational Lapse to CMS's Statutory Payment Obligations

A lapse in HQR system maintainer services — or a transition to an insufficiently prepared contractor — would expose CMS to significant legal and operational risks, including:

Failure to issue accurate and timely Medicare payments and associated penalties in accordance with statutory requirements may result in increased provider disputes, administrative appeals, and heightened congressional oversight, thereby exposing CMS to significant operational, legal, and reputational risk.

Disruption to the Hospital Star Ratings publication cycle on Medicare.gov, affecting public transparency and CMS's commitments to beneficiaries and stakeholders.

Loss of data integrity across the 43+ million production files annually processed by the HQR system, with downstream effects on measure calculations, performance reports, and public reporting accuracy.

Breach of interagency data-sharing commitments with the VHA and DoW, whose facility performance data is dependent on continuous, uninterrupted HQR operations.

5. Market Research:

CCSQ/ISG, in coordination with OAGM, conducted an extensive analysis of existing ISG contracts to identify capable vendors who could fulfill this requirement during the bridge period. The analysis concluded that, while various firms perform software development work on other ISG contracts, that development is system-specific and not transferable to the HQR environment. The significant variability in system infrastructure, tooling, technology stack, and program requirements between ISG contracts — particularly between PaaS-based systems and custom-built applications such as HQR — means that experience on other ISG contracts does not confer the system-specific expertise required to serve as the HQR system maintainer.

Furthermore, the size and scope of the HQR system maintainer requirements are not suitable for incorporation into another CCSQ contract's Performance Work Statement. The HQR

program's scale — spanning 10 quality reporting programs, 71 submission windows, and over 10,000 participating facilities — warrants a dedicated, standalone contract.

No other vendor was identified through market research as capable of assuming the HQR system maintainer role at the required level of quality within the timeframe of the bridge contract without incurring unacceptable operational risk to CMS.

6. Procurement history:

The HQR system maintainer contract (HQR II) was originally scheduled to expire in June 2025, with system maintainer requirements to be transferred to a competitively awarded successor contract — the HQR Enhancement & Advancement Technologies (HEAT) contract. Upon award of the HEAT contract in June 2025, CMS received multiple Government Accountability Office (GAO) protests that persisted through February 2026. The protests prohibited CMS from lifting the stop-work order on the HEAT contract, necessitating multiple sequential extensions of system maintainer services under the HQR II contract pursuant to FAR 52.217-8.

As of August 2026, no further extensions are available under the HQR II contract, as the cumulative extension period has reached the FAR 52.217-8 statutory limit. Simultaneously, the passage of time since the original HEAT solicitation — combined with shifts in federal policy, technological advancements, and federal staffing changes — has rendered the original HEAT Performance Work Statement (PWS) and solicitation materially obsolete. CMS/CCSQ has therefore determined it is necessary to revise the HEAT requirements and re-issue the solicitation on the GSA Multiple Award Schedule (MAS) under a fully competitive process.

This sole-source bridge contract represents the best available mechanism for the Government to maintain uninterrupted HQR system operations during the period required to revise, solicit, evaluate, and award a re-competed HEAT contract. This approach ensures continuity of critical operations while minimizing performance, schedule, and transition risks that could otherwise jeopardize mission requirements and statutory obligations.

a. Contractor:

Contract or Order Number: HHSM500201600009B/75FCMC21F0002

Vendor: Bellese Technologies LLC

Award Date and Period of Performance: Award 6/21/21, PoP 8/7/21-5/6/26

Value (including all options): \$101,292,406.92

Competitive Status: Full and Open Competition

Authority formerly relied upon if Not Competed: N/A

7. Actions to Foster Competition for Future, Similar Requirements: This limited-sources justification is a direct consequence of protracted GAO protest proceedings against the HEAT competitive re-compete, which prevented CMS from executing a planned, orderly transition to a competitively selected successor contractor. The Agency has not chosen to limit competition;

rather, extraordinary circumstances — multiple sequential protests and the resulting stop-work order — have eliminated the available competitive options during the bridge period.

The bridge contract is structured to minimize the period during which competition is restricted:

The Base Period of 12 months provides the minimum time necessary for CCSQ to revise the HEAT Acquisition Plan, PWS, and Independent Government Cost Estimate (IGCE), conduct thorough market research, and issue a competitive solicitation on GSA MAS.

One 6-month Option Period (Option to Extend) provides scheduling flexibility in the event the HEAT competition is concluded early or encounters additional procurement delays.

CMS remains committed to re-establishing full and open competition for long-term HQR system maintainer services through the HEAT re-solicitation and will not extend or recomplete this bridge contract beyond its authorized term.

8. Additional Information or Other Facts: In addition to the foregoing, the following additional facts support the sole-source determination:

GSA Schedule Pricing: The proposed contractor holds a current GSA Multiple Award Schedule contract. Pricing under this bridge contract will be drawn from established Schedule rates, ensuring that the Government benefits from competition-derived pricing even within the limited-source procurement.

Prior Competitive Awards: Bellese Technologies was competitively selected as the HQR system maintainer under the HQR II contract. The current sole-source action does not reflect a failure of competition but rather an extraordinary circumstance created by sustained protest activity against the competitive successor contract.

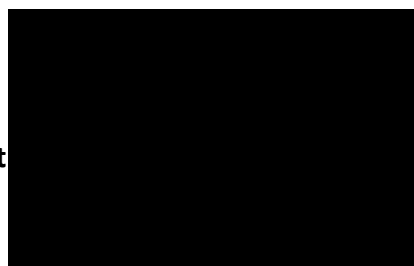
Best Value: Given the operational stakes — statutory Medicare payment obligations, public reporting timelines, and interagency data-sharing commitments — the cost of a failed or disrupted transition would far exceed any savings achievable by awarding this short-term bridge contract to an unproven contractor. Continuity of the incumbent contractor represents best value to the Government.

9. Technical or Requirements Program Management Certification: I certify that the supporting data under my cognizance which are included in the justification are accurate and complete to the best of my knowledge and belief.



Contracting Officers Representative

Signature & Date



10. Best Value Determination:

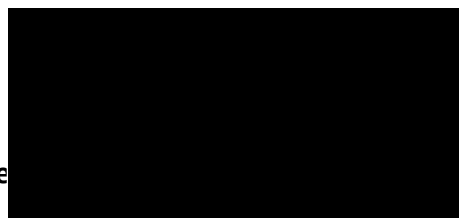
As the Contracting Officer, I hereby determine that the anticipated price to the Government for this contract action represents the best value for the Government. Proposed pricing is based on established, competition-derived GSA Multiple Award Schedule rates under Contract No. GS-35F-356DA and is reasonable given the requirement for immediate, uninterrupted performance of highly specialized HQR system maintainer services with no transition-in period. The scope includes full operations and maintenance, 26 agile development sprints annually, support for ten quality reporting programs, and mission-critical outputs tied to Medicare payments and public reporting, all of which necessitate continuity without performance degradation. Market research determined that no other vendors could meet these requirements within the bridge timeframe due to the system's unique architecture and operational dependencies, and prior competitive award history further supports price reasonableness. While no additional discounts are currently identified beyond GSA Schedule pricing, the Government will seek any available order-level price reductions during negotiations to ensure continued price competitiveness.

11. Contracting Officer (CO) Certification and Approval: I hereby certify that this justification is accurate and complete to the best of my knowledge.



Contracting Officer

Signature & Date:

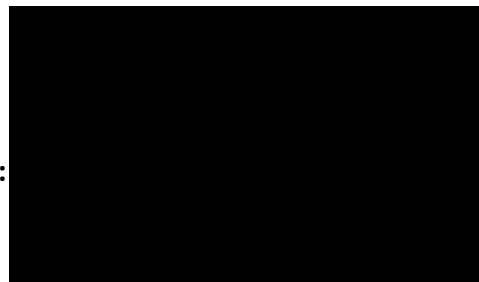


12. Division Director/SBC Director Certification and Approval: I reviewed this justification and find that it adequately supports a sole source acquisition.



Division Director

Signature & Date:



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The following Reviews and Approvals will ordinarily begin on a separate page at the end of the justification to allow minor, administrative changes to be made during the review process; delete any individual reviews/approvals not applicable.

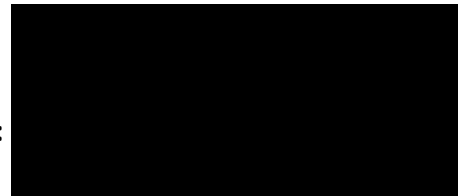
ELEVATED REVIEWS & APPROVALS

13. OMAS Advocate for Competition Review and Approval: I reviewed this justification and find that it adequately supports a sole source acquisition.



Procurement Analyst

Signature & Date:



FINAL APPROVAL

14. OMAS Head of the Contracting Activity (HCA) Review and Approval: I reviewed this justification and find that it adequately supports a sole source acquisition.



Head of the Contracting Activity

Signature & Date:

